



BOTSWANA NATIONAL HEALTH QUALITY STANDARDS FOR HOSPITALS

44. Psychiatric Rehabilitation Care

NAME OF HEALTHCARE FACILITY: _____

TYPE OF SURVEY: _____

DATE OF SURVEY: _____

DOCUMENT COMPLETED BY (Title and Name)

INTERVIEWEE (Name and Designation)

This Standard Assessment Manual (SAM) provides a means to record the level of compliance achieved for each criterion (NA NC PC C), comments explaining why compliance has not been achieved and actions required to achieve compliance. Each criterion has a guideline statement which provides clarification of the requirements for compliance.

In summary, criteria are scored as follows:

- 1. Compliant (C)** means the condition required is substantially met. Evidence of compliance should be present in a tangible and/or observable form, e.g. documentation, physical items, etc.
- 2. Partially Compliant (PC)** means the condition required is not totally met, but there is progress towards compliance.
- 3. Non-compliant (NC)** means there is no observable progress or minimal progress towards complying with the requirement specified in the criterion.
- 4. Not applicable (NA)** means the criterion will not be scored because the service is either not provided, or not required at the particular level of care the criterion is designed to measure.

Please circle the compliance rating assigned to each criterion. Explanatory comments should be provided for each criterion which does not receive a Compliant (C) score.

Documents Checked

Surveyor:

Surveyor:



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Overview of Psychiatric Rehabilitation Care

A healthcare organisation's main purpose is patient care. Providing the most appropriate care in a setting that supports and responds to each patient's unique needs requires a high level of planning and coordination.

Certain activities are basic to patient care, including planning and delivering care to each patient, monitoring the patient to understand the results of the care, modifying care when necessary and completing the follow-up.

Many medical, nursing, pharmaceutical, rehabilitation and other types of healthcare providers may carry out these activities. Each provider has a clear role in patient care. Credentialing, registration, law and regulation, an individual's skills, knowledge and experience and organisational policies or job descriptions determine that role. The patient, the family or trained caregivers may carry out some of this care.

A plan for each patient is based on an assessment of needs. Care may be preventive, palliative, curative or rehabilitative and may include the use of medication, supportive therapies, or a combination of these. A plan of care is not sufficient to achieve optimal outcomes unless the delivery of the services is coordinated, integrated and monitored.

Continuity of care:

From entry to discharge or transfer, several departments, services and different health care providers may be involved in providing care. Throughout all phases of care, patient needs are matched with appropriate resources within and, when necessary, outside the organisation. This is accomplished by using established criteria or policies that determine the appropriateness of transfers within the organisation.

Processes for continuity and coordination of care among psychiatrists and medical practitioners, nurses and other health care providers must be implemented in and between all services.

Leaders of various settings and services work together to design and implement the required processes to ensure coordination of care.



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Standards

44.1 Coordination of Patient Care

44.1.1 Standard

During all phases of care, there are qualified individuals responsible for the patient's care.

Intent of 44.1.1

The individuals who bear overall responsibility for the patient's care or a particular phase of care are identified in the patient's record or in a manner that is made known to the personnel.

44.1.1 Criteria

44.1.1.1	An appropriately qualified individual has clearly defined responsibilities and accountability for all aspects of the service.	Comment:
Severity: Very Serious		
Category: Efficiency		
NA NC PC C		
Guideline statement: The unit manager's job description and qualifications will be reviewed during the personnel record audit.		

44.1.1.2	The individuals responsible for the patient's care are designated.	Comment:
Severity: Very Serious		
Category: Legality		
NA NC PC C		
Guideline statement: The focus is on the core care-givers i.e. medical practitioners and nurses. Evidence is required in the form of personnel allocation lists, duty rosters and off-duty lists. It is important to assess the availability of medical practitioners, especially after-hours, including weekends and public holidays.		

44.1.1.3	The individuals responsible for the patient's care are qualified.	Comment:
Severity: Very Serious		
Category: Legality		
NA NC PC C		
Guideline statement: The lists mentioned above must be examined for evidence of adequate numbers of the various categories of personnel in relation to the type of patients, acuity levels, bed occupancy levels etc. It is important that there are sufficient numbers of psychiatric qualified personnel available on each shift – at least one per shift. Proof of current registration with the relevant authority, where applicable, must be available.		

44.1.1.4	The individuals responsible for the patient's care are identified and made known to the patient and other personnel.	Comment:
Severity: Very Serious		
Category: Efficiency		
NA NC PC C		



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Guideline statement: Name badges should be worn by all personnel members at all times. The process for obtaining informed consent should include informing the patient of the name of the individual who will be performing the procedure for which consent is being obtained. Patient interviews will reveal whether personnel introduced themselves to patients.

44.1.2 Standard

Clinical practice guidelines are used to guide patient care and reduce unwanted variation.

Intent of 44.1.2

Clinical practice guidelines provide a means for improving quality and assist practitioners in making clinical decisions. Consideration should be given to providing guidelines for high risk, high volume and high cost conditions as these are the areas that represent the highest risk to patients and the organisation. In addition, guidelines should be available for conditions which are rarely seen but may have severe consequences for patients if misdiagnosed or mismanaged.

Guidelines are found in the literature under many names including practice parameters, practice guidelines, patient care protocols, standards of practice, care pathways, etc. Regardless of the source, the scientific basis of guidelines should be reviewed and approved by the clinical leaders and clinical practitioners before implementation. This will ensure that they meet the criteria established by the leaders and are adapted to the community, patient needs and organisation resources. Once adopted, guidelines should be reviewed on a regular basis to ensure their continued relevance.

44.1.2 Criteria

44.1.2.1 Severity: Very Serious Category: Patient Care NA NC PC C	Evidence-based clinical practice guidelines relevant to the patients and services of the organisation are available to guide patient care processes.	Comment:
Guideline statement: Sometimes such evidence is available for nursing practices only, in which case this will be scored PC. In the absence of guidelines this criterion will be scored NC.		

44.1.2.2 Severity: Serious Category: Patient Care NA NC PC C	Organisational and clinical leaders set criteria to select clinical practice guidelines.	Comment:
Guideline statement: Every effort should be made to identify and adopt guidelines that have been rigorously and scientifically developed, internationally accepted and adapted for local use.		

44.1.2.3 Severity: Very Serious Category: Patient Care NA NC PC C	Implemented guidelines define and establish admission criteria for the rehabilitation service.	Comment:
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Guideline statement: For optimal patient care and cost-effectiveness of service delivery, patients should only be admitted to specialist patient care units when they require the services offered by these units. To ensure that only patients requiring these services are admitted to these units, admission criteria should be established and applied to each potential admission. Admission criteria should be developed in collaboration with all relevant services, i.e. organisations who will refer to the specialist unit, the specialist unit themselves and the organisations to which patients will be transferred from the specialist unit. It may be appropriate to include administrative personnel in the process.

44.1.2.4	The implementation of guidelines is monitored as part of a structured clinical audit.	Comment:
Severity: Very Serious		
Category: Efficiency		
NA NC PC C		

Guideline statement: Clinical audits cannot be done on ALL guidelines, but evidence must be available that the management of high cost, high risk, high volume and problem-prone conditions are considered for auditing. If guidelines are available but structured clinical audits are not done, this criterion is scored NC. Sometimes such evidence exists for nursing practices only, in which case this will be scored PC.

44.1.2.5	Guidelines are reviewed on a regular basis and updated when necessary.	Comment:
Severity: Very Serious		
Category: Efficiency		
NA NC PC C		

Guideline statement: Organisational policy should define the frequency with which guidelines are to be reviewed to ensure they reflect current practice. This should include updating of guidelines following major changes in best practice advice as well as routine review.



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44.1.3 Standard

The delivery of services is integrated and coordinated amongst care providers.

Intent of 44.1.3

The coordination of patient care depends on the exchange of information between the multidisciplinary team. This can be through verbal, written or electronic means according to appropriate policies determined by the organisation. Clinical leaders should use techniques to better integrate and coordinate care for their patients (for example, team-delivered care, multi-departmental patient care rounds, combined care planning forums, integrated patient records, case managers). The process for working together will be simple and informal when the patient's needs are not complex.

The patient, family and others are included in the decision process when appropriate.

The patient's record contains a history of all care provided by the multidisciplinary team and is made available to all relevant caregivers who are authorised to have access to its content.

44.1.3 Criteria

44.1.3.1	Care planning is integrated and coordinated among all care providers.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Evidence of team-delivered care, multi-departmental patient care rounds, combined care planning forums, integrated patient records, case managers, etc. will be required.		

44.1.3.2	The patients' clinical records are completed according to organisation policy.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: This refers to the relevant forms/documents that should be present in the patient's record, as well as the completeness of entries made in the record. For this purpose, the results of patient record audits should be taken into account. Therefore, the score of this criterion needs to reflect the aggregated average score of all relevant criteria in this document. If the doctors' (or other health care practitioners') clinical notes are not available in the patient records, this criterion is scored PC.		

44.1.3.3	The patient's records are up to date to ensure the transfer of the latest information between care providers.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Clinical professionals (medical practitioners, nurses, therapeutic support services, social workers, etc.) form part of the multidisciplinary/interdisciplinary team and should all contribute to the patient record. Records of multidisciplinary/interdisciplinary team meetings should be documented as evidence of integrated care. All patient care information will be obtained from notes made by each member of the above team. The clinical involvement of medical practitioners must be measured in all relevant sections of the service. The notes in the patient record will indicate whether the patient has been assessed (initial, repeat, follow-up) by the relevant care providers at intervals appropriate to the patient's condition and whether these assessments have been recorded satisfactorily. Records must be available to all care providers to enable them to make the necessary notes and/or check information. Compliance will be verified during the patient record audit. If the notes of doctors and other health care professionals are not available because they are kept separately in another location, this criterion will be scored PC.		



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44.1.3.4	Information exchanged includes the patient's health status.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: This includes both the patient's general health status and mental health status. Compliance will be verified during the patient record audit.		

44.1.3.5	Information exchanged includes a summary of the care provided.	Comment:
Severity: Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: If the doctors' (and other health care practitioners') notes are not available in the patient record, this criterion is scored PC. Compliance will be verified during the patient record audit.		

44.1.3.6	Information exchanged includes the patient's progress.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during the patient record audit.		

44.1.3.7	The author can be identified for each patient record entry.	Comment:
Severity: Very Serious		
Category: Legality		
NA NC PC C		
Guideline statement: In order to ensure that appropriately qualified individuals provide and record their activities related to patient care, names, signatures and designations must be available and the authors must be identifiable. Methods of verifying signatures and designations could include: • Names and designations printed below signatures • Stamps indicating names and designations • Specimen signature lists in the patient records or in the relevant department The specimen signature lists must be up to date and include all relevant signatures and initials. Specimen signatures and initials must match the usual signatures and initials used in patient records. If signature lists are used, these should be archived for future reference. Compliance will be verified during the patient record audit. If the signatures and designations cannot be matched to any of the systems mentioned above, this criterion will be marked PC or NC based on audit results.		

44.1.3.8	The date and time of each patient record entry can be identified.	Comment:
Severity: Very Serious		
Category: Patient Care		
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Guideline statement: Records must follow a chronological order to allow for a logical flow of information. Compliance will be verified during the patient record audit.

44.2 Assessment of Patients

44.2.1 Standard

All patients cared for by the organisation have their healthcare needs identified through an established assessment process within appropriate time frames.

Intent of 44.2.1

When a patient enters a ward or department, the specific information required and the procedures for obtaining and documenting it depend on the patient's needs and on the setting in which care is being provided.

The organisation defines, in writing, the scope and content of assessments to be performed by each clinical discipline within its scope of practice and applicable laws and regulations.

These findings are used throughout the care process to evaluate patient progress and understand the need for reassessment. It is essential that assessments are documented well and can be easily retrieved from the patient's record.

The healthcare organisation determines the time frame for completing assessments. This may vary in the different settings within the organisation.

44.2.1 Criteria

44.2.1.1	Written policies and procedures for assessing patients and their families on admission and during ongoing care are implemented.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: A policy must be available which defines the scope of practice for each discipline, together with specific information required, procedures to be carried out and all documentation to be completed during the assessment of patients and their families. The provision of specifically designed assessment forms for each relevant discipline is considered as evidence of the implementation of such a policy. Where the required information, according to the patient's presenting complaints/condition is not documented, the score will be adjusted accordingly. Compliance will be verified during the patient record audit.		

44.2.1.2	Only those individuals permitted by applicable laws, regulations and professional registration perform the assessments.	Comment:
Severity: Very Serious		
Category: Legality		
NA NC PC C		
Guideline statement: Compliance will be verified during the patient record audit, which will focus on the legitimacy of signatures and the identification of designations/qualifications. The absence of notes made by the doctor in the patient record, or other relevant clinical personnel other than nursing personnel will affect the outcome of the audit process.		



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44.2.1.3	The scope and content of assessment by each discipline is defined.	Comment:
Severity: Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: It is accepted that the scope and content of assessment for each discipline is defined if the organisation makes use of standardised formats of clinical assessment forms for specific use by different disciplines. The absence of medical notes or other relevant clinical personnel other than nursing personnel in the patient record will affect the outcome of the audit process. Compliance will be verified during the patient record audit.		

44.2.1.4	Assessments are performed within appropriate time frames and are comprehensively documented in the patient's records according to organisation policy.	Comment:
Severity: Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Time frames for the assessment of patients should be established for each discipline, for example, there should be an indication of which cases are urgent/or not priority cases. It is important to note that this criterion does not refer to the length of time that it takes to perform the assessment, but the time within which the initial assessment should be commenced and/or completed. These time frames should be stated in the policy framework. Compliance will be verified during the patient record audit.		

44.2.1.5	The date and time of assessment can be identified.	Comment:
Severity: Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during the patient record audit.		



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44.2.2 Standard

Each patient has an initial assessment which complies with current policies, procedures and guidelines.

Intent of 44.2.2

The initial assessment of a patient is critical for the identification of the needs of the patient and initiation of the care process. Patients' social, cultural and family status are important factors that can influence their response to illness and care. Families can be of considerable assistance in these areas of assessment and in understanding the patient's wishes and preferences. Financial factors should be assessed as part of the social assessment, particularly when the patient and his/her family will be responsible for the cost of all or a portion of the care.

A functional and nutritional assessment allows for the patient to be referred for specialist care if necessary.

Certain patients may require a modified assessment, for example, very young patients, the frail or elderly, those terminally ill or in pain, patients suspected of drug and/or alcohol dependence and victims of abuse and neglect. When appropriate, the assessment process should be modified to respect local cultural practices.

The outcome of the patient's initial assessment should result in an understanding of the patient's medical, nursing and rehabilitation needs so that care and treatment can begin.

Planning for discharge should be commenced during this initial assessment process.

When the medical assessment was conducted by a different healthcare organisation, a legible copy of the findings should be filed in the patient's record. Any significant changes in the patient's condition since this assessment should be recorded.

44.2.2 Criteria

44.2.2.1	Each patient admitted has an initial assessment which meets organisation policy.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Documented evidence in the form of a detailed policy and procedure or guideline is provided. To ensure coordination of patient care, all members of the team should be included in the formulation of this policy framework. Compliance will be verified during the patient record audit.		

44.2.2.2	The initial assessment includes a health history.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance is measured in the relevant areas in the patient's records. This criterion will be scored PC if medical practitioners' notes or notes by relevant clinical personnel, other than nurses, are not available. Compliance will be verified during the patient record audit.		



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44.2.2.3	The initial assessment includes a physical examination.	Comment:
Severity: Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: If the doctors' notes are not available this criterion is scored NC. Compliance will be verified during the patient record audit.		

44.2.2.4	The initial assessment includes a functional and nutritional examination, where applicable.	Comment:
Severity: Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during the patient record audit.		

44.2.2.5	The initial assessment includes a social and economic assessment, where applicable.	Comment:
Severity: Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during the patient record audit.		

44.2.2.6	The initial assessment includes a psychological assessment, where applicable.	Comment:
Severity: Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during the patient record audit.		

44.2.2.7	The initial assessment includes a cultural assessment, where applicable.	Comment:
Severity: Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during the patient record audit.		

44.2.2.8	The initial assessment results in a diagnosis and the formulation of an individualised plan of care for the patient.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during the patient record audit.		



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44.2.2.9	The organisation identifies patients in pain during the initial assessment process.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during the patient record audit.		

44.2.2.10	The organisation identifies those patient populations and special situations for which the initial assessment process is modified.	Comment:
Severity: Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Documented evidence in the form of a detailed policy and procedure or guideline is provided. To ensure coordination of patient care, all members of the team should be included in the formulation of this policy framework, especially doctors and nurses. Compliance will be verified during the patient record audit.		

44.2.2.11	Those special patient populations receive individualised assessments.	Comment:
Severity: Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during the patient record audit.		

44.2.2.12	The findings of assessments performed outside the organisation are verified on admission.	Comment:
Severity: Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during the patient record audit.		

44.2.2.13	Any significant changes in the patient's condition since the assessment performed outside of the organisation are noted in the patient's record.	Comment:
Severity: Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during the patient record audit.		

44.2.2.14	A process is in place to identify needs for discharge planning at the initial assessment.	Comment:
Severity: Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during the patient record audit.		



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44.2.3 Standard

Pre-and post-anaesthetic assessments are documented.

Intent of 44.2.3

Psychiatric patients sometimes receive anaesthesia for electroconvulsive therapy. The preoperative anaesthesia assessment determines if the patient is a suitable candidate for the planned anaesthetic. The clinical assessment and results of investigations must be available to the doctor performing the assessment.

A patient's post-procedural care in recovery and at ward level is related to the procedure.

44.2.3 Criteria

44.2.3.1	The patient's pre-anaesthetic medical assessment to determine fitness for anaesthesia is documented.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Organisational policy must define who may perform this assessment and what the pre-anaesthetic assessment should include. The results of this assessment must be recorded by the individual who performs the assessment. Because the criterion refers to a medical assessment, it is scored NC if the patient record does not reflect notes by the person responsible for administering the anaesthetic. If only the nurses' assessment notes are available in the patient record, this criterion will be scored NC. Compliance will be verified during patient record audit.		

44.2.3.2	The initial medical assessment of patients is documented before anaesthesia.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Organisational policy must define who may perform this assessment and what the medical assessment should include. The results of this assessment must be recorded by the individual who performs the assessment. Because the criterion refers to a medical assessment, it is scored NC if the patient record does not reflect notes by a medical practitioner. If only the nurses' assessment notes are available in the patient record, this criterion will be scored NC. Compliance will be verified during patient record audit.		

44.2.3.3	Patients have the results of diagnostic tests recorded before anaesthesia.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during patient record audit.		

44.2.3.4	The name of the medical practitioner responsible for the procedure is documented.	Comment:
Severity: Very Serious		
Category: Legality		
NA NC PC C		
Guideline statement: Compliance will be verified during patient record audit.		



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44.2.3.5	The patient's physiological status is monitored during the post-procedural period.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: This includes monitoring in the recovery room and in the ward following transfer. Compliance will be verified during patient record audit.		

44.2.4 Standard

Healthcare professionals responsible for patient care collaborate to analyse and integrate assessment information.

Intent of 44.2.4

Patients benefit most when the personnel responsible for their care work together to analyse the assessment findings and to combine this information into a comprehensive picture of his or her condition. From this collaboration, the patient's needs are identified, the order of their importance is established, and care decisions are made.

44.2.4 Criteria

44.2.4.1	Assessment findings are documented in the patient's record and are readily available to those responsible for the patient's care.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: There must be evidence that all clinical professionals dealing with the patient have contributed to the patient's record. If the doctors' (or other health care practitioners') clinical notes are not available in the patient records, this criterion is scored PC. Compliance will be verified during the patient record audit.		

44.2.4.2	Patient assessment data and information are analysed and integrated by those responsible for the patient's care.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Those involved in the patient's care form the multidisciplinary team. The constitution of the team will vary for each patient and may include representatives from the medical, nursing, social work, psychology, occupational therapy, physiotherapy and other disciplines, departments or services. Information from all disciplines involved in the patient's care should be considered when developing the treatment plan. Compliance will be verified during the patient record audit.		

44.2.4.3	Patient needs are prioritised based on assessment results.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		



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Guideline statement: Compliance will be verified during patient record audit.

44.2.4.4	The patient and/or his or her family participate in the decisions regarding the priority needs to be met.	Comment:
Severity: Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during patient interviews and/or the patient record audits.		



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44.3 Patient Care

44.3.1 Standard

Adequate resources are available for the provision of safe care to patients in the service.

Intent of 44.3.1

In order to provide safe patient care, each unit requires adequate resources. The physical facilities required include adequate office accommodation for personnel, sluice rooms which are hygienically clean, treatment and dressing rooms, and adequate storage space for clean linen.

Counseling and treatment rooms, group rooms, a dining room, an out-door area, structured activity programmes and condition-appropriate games should also be available.

Cleaning equipment is safely stored in a room or cupboard expressly for this purpose. No cleaning equipment should be stored in areas where clean linen, medical supplies or food are stored. There are adequate toilet and bathing facilities for the number of patients in the ward as determined by country specific legislation.

There is adequate lighting and ventilation. Nurse call systems are available at bedsides and in bathrooms and toilets and is connected to the emergency power supply.

There is at least one oxygen and one vacuum point for every 2 beds. Where there is no piped oxygen and vacuum supply, there are mobile oxygen and vacuum pumps. All necessary fittings for oxygen and suction are in place and working. Each bed is serviced by at least one electrical socket outlet. Each ward is provided with a socket outlet which is connected to the emergency power supply.

Every patient care area must have access to resuscitation equipment within one minute of any patient collapsing that includes as a minimum:

- Defibrillator or automated external defibrillator (AED) with adult and paediatric (where applicable) paddles/pads available within three minutes of any patient collapsing
- ECG monitor
- CPR board
- Suction apparatus, hard and soft suction catheters and suction tubing
- A bag-mask manual ventilator (Ambubag) or equivalent
- A range of endotracheal tubes and two laryngoscopes, with a range of straight and curved blades, spare batteries and spare globes where applicable
- An introducer/stylet for endotracheal intubation
- A syringe to inflate the endotracheal tube cuff
- A laryngeal mask airway
- Oral airways
- Equipment to perform an emergency cricothyroidotomy
- Medication for cardiac and respiratory arrest, coma, fits and states of shock (including paediatric doses where applicable)
- Appropriate consumables for intravenous therapy and medication administration and volume expanders (including paediatric sizes where applicable)

44.3.1 Criteria

44.3.1.1	Patient and personnel accommodation in the service is adequate to meet patient care needs.	Comment:
Severity: Serious		
Category: Patient Care		
NA NC PC C		



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Guideline statement: There should be adequate office space for personnel; clean, hygienic sluice rooms, treatment and dressing rooms; adequate, tidy linen and equipment storage facilities; suitable lighting and ventilation; adequate, hygienic bathroom and toilet facilities for the number of patients, as well as the requirements mentioned below.

44.3.1.2	There is evidence that equipment is maintained in accordance with the policies of the organisation.	Comment:
Severity: Very Serious		
Category: Pat & Staff Safety		
NA NC PC C		

Guideline statement: This refers to medical equipment and not to furniture such as beds.

44.3.1.3 CRITICAL	Resuscitation equipment is available in accordance with the policies of the organisation and includes a) – m) in the standard intent above.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		

Guideline statement: Resuscitation equipment and documented evidence of the checking thereof in the unit, according to the organisation's resuscitation policy must be available. Checking must include the identification of expiry dates on consumables such as airways and endotracheal tubes. Documented evidence of this checking is required. A resuscitation trolley must be available at the point of need within one minute. In addition, there must be access to a defibrillator or automated external defibrillator (AED) within three minutes of any patient collapsing.
The 40/80 rule for compliance will apply.

44.3.1.4	Each patient has access to a nurse call system at all times.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		

Guideline statement: A functional nurse call system must be available at each patient's bed and in the ablution facilities. Random tests can be conducted to assess function and response. Assessment will be made on the response from patient interviews and patient complaints.
This criterion will be scored accordingly, for example, NC if there is no functional call system and PC if nurses do not respond.

44.3.1.5	Oxygen and vacuum supplies meet patient care requirements.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		

Guideline statement: There should be either piped oxygen/vacuum or mobile oxygen cylinders and vacuum pumps in working order to meet the patients' needs. This includes the availability of a sufficient number of functional flow-meters.
Any evidence of infection control deficiencies for example, stagnant water in humidifiers/vacuum bottles will be linked to Prevention and Control of Infections.



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44.3.1.6	When patients receive oxygen from a cylinder, the cylinder pressures are monitored according to organisation policy.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: This applies to situations when patients are given oxygen by cylinder, for example, where no piped oxygen is available or in the event that the piped oxygen system fails. The cylinder pressure indicates the remaining contents of the cylinder. Monitoring the contents will alert personnel to the need for the cylinder to be replaced and prevent the interruption of the patient's oxygen supply.		

44.3.1.7	Electricity and water is available in accordance with the policies and arrangements of the organisation.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Ward/unit personnel should have knowledge of routine arrangements and the contingency plans.		

44.3.2 Standard

The care provided to each patient is planned and written in the patient's record.

Intent of 44.3.2

A single, integrated plan is preferable to the entry of a separate care plan recorded by each healthcare professional. The care plan should include the care to be delivered and the intended goals of care.

Collaborative care and treatment team meetings or similar patient discussions are recorded. Diagnostic and other procedures must be ordered by individuals qualified to do so. These orders must be easily accessible if they are to be acted on in a timely manner. Locating orders on a common sheet or in a uniform location in patient records facilitates the correct understanding and carrying out of orders. The reason for requesting diagnostic imaging or laboratory tests should be recorded if this reason will be required for interpretation of the results.

An organisation decides:

- Which orders must be written rather than verbal
- Who is permitted to write orders
- Where orders are to be located in the patient record

The method used must respect the confidentiality of patient care information for example, order books must not be kept in an area where they can be seen by other patients or visitors and verbal orders, including those given over the telephone, should be taken in an area where they cannot be overheard.

44.3.2 Criteria

44.3.2.1	The care for each patient is planned, provided and noted in the patient's record.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during the patient record audit.		



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44.3.2.2	The patient's response to care and therapeutic interventions is documented in the patient record.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during the patient record audit.		

44.3.2.3	Meetings to discuss patient care/treatment regimes are held by the multidisciplinary team at regular intervals as determined by organisational policy and documented in the patient's record.	Comment:
Severity: Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during the patient record audit.		

44.3.2.4	All procedures and diagnostic tests ordered and performed, are documented in the patient's record.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: This documented evidence may include nurses' and doctors' notes as well as printed reports, which are filed in the patient record. Compliance will be verified during the patient record audit.		

44.3.2.5	The results of procedures and diagnostic tests performed are available in the patient's record.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: This criterion will be scored PC if the results are not initialled by the doctor, the treatment plan does not indicate action taken, or the nursing notes do not indicate that the doctor was informed. Compliance will be verified during the patient record audit.		

44.3.2.6	Orders are found in a uniform location in patient records.	Comment:
Severity: Moderate		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during the patient record audit.		

44.3.2.7	Only those permitted to write orders do so.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during the patient record audit.		

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44.3.2.8	Reassessments are performed at appropriate, regular intervals and following a change in the patient's condition, and documented in the patient's record.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Patients should be reassessed according to clinical need. This may be continuous, hourly or daily (as a minimum), depending on the clinical condition of the patient. Any deterioration in a patient's condition should result in a formal reassessment by an appropriately trained clinician – often a nurse in the first instance followed by a doctor if clinically indicated. The nature of these reassessments and the processes to be followed should be defined in organisation or departmental policy. If the doctors' (or other health care practitioners') clinical notes are not available in the patient's records, this criterion will be scored PC. Compliance will be verified during the patient record audit.		

44.3.2.9	Care plans are revised when necessary in response to the findings of reassessments.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during the patient record audit.		

44.3.3 Standard

Each patient participates in a structured treatment plan.

Intent of 44.3.3

Each patient has psychotherapeutic interviews with an appropriately qualified person to meet his/her needs.

There is a structured therapeutic environment which allows for group therapy, occupational therapy, or music or art therapy as required by individual patients.

44.3.3 Criteria

44.3.3.1	There is evidence of regular psychotherapeutic interviews as indicated by the programme and individual patient's needs.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during the patient record audit.		

44.3.3.2	There is documented participation in a structured therapeutic programme six to seven hours per day.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during the patient record audit.		



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44.3.3.3	There is a range of therapeutic activities available, according to the identified needs of the patient.	Comment:
Severity: Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: The unit should document all therapeutic activities available to patients together with relevant information of how to access these activities, for example, schedules of when the activities are available, contact details of service providers, activities offered by organisations outside the organisation for example, local NGO's or other service providers, referral documents, etc.		

44.3.3.4	There is documented participation of the patient with his or her family or significant other(s) in group therapy, as appropriate.	Comment:
Severity: Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during the patient record audit.		

44.3.3.5	The patient has the least restrictive environment possible, with any restrictions placed upon him/her written into the treatment plan.	Comment:
Severity: Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during the patient record audit and the survey visit to the unit.		

44.3.3.6	Implemented policies and procedures guide the freedom of movement within the facility and weekend or other leave from the organisation.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during the patient record audit.		



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44.3.4 Standard

Compassionate care is provided to the dying and to those in pain.

Intent of 44.3.4

While pain may be a part of the patient experience, unrelieved pain has adverse physical and psychological effects. The patient's right to appropriate assessment and management of pain is respected and supported. The organisation has processes to:

- Identify patients with pain during initial assessment and reassessment
- Communicate with, and provide education for, patients and families about pain management in the context of their personal, cultural and religious beliefs
- Educate healthcare providers in pain assessment and management.

Dying patients have unique needs for respectful, compassionate care. Concern for the patient's comfort and dignity guides all aspects of care during the final stages of life. To accomplish this, all personnel are made aware of the unique needs of patients at the end of life. These needs include treatment of primary and secondary symptoms; pain management; response to the psychological, social, emotional, religious and cultural concerns of the patient and family; and involvement in care decisions.

44.3.4 Criteria

44.3.4.1	The assessment process makes provision for patients in pain to be identified.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: This requirement should be included in the assessment policy and procedure for the organisation and must include methods for identifying pain in non-communicating and intellectually disabled patients. Compliance will be verified during patient record audit.		

44.3.4.2	Patients in pain receive care according to pain management guidelines.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: There must be evidence that all clinical professionals dealing with the patient's pain management have contributed to the patient's record. Monitoring of the effectiveness of pain management must be recorded. Evidence could include the documentation of pain using an individualised assessment form and pain scoring charts. Pain management continues with the prescription and administration of medication for pain as well as the follow-up assessment after treatment. These records will also be evaluated. Compliance will be verified during the patient record audit.		

44.3.4.3	There is evidence that the effectiveness of pain and symptom management is monitored.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during patient record audit.		



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44.3.4.4	Patients and families are educated about pain and pain management.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during the patient record audit and patient interviews.		

44.3.4.5	The organisation has processes to educate health professionals in assessing and managing pain.	Comment:
Severity: Very Serious		
Category: Efficiency		
NA NC PC C		
Guideline statement: Training must include the identification of pain in non-communicating and intellectually disabled patients. Documented evidence of personnel training will be required.		

44.3.4.6	Personnel provide respectful and compassionate care to dying patients.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: This includes the organisation respecting the need of the physically ill or dying patient to be transferred to another suitable facility/unit. Compliance will be verified during patient record audit and patient interviews.		



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44.3.5 Standard

Policies and procedures guide the care of high-risk patients and the provision of high-risk services.

Intent of 44.3.5

Some patients are considered "high-risk" because of their age, condition or the critical nature of their needs. Patients in rehabilitative care are commonly in this group as they may not speak for themselves, understand the care process or participate in decisions regarding their care.

Policies and procedures are important for personnel to understand high-risk patients and services, and to respond in a thorough, competent and uniform manner. The clinical and managerial leaders take responsibility for identifying the patients and services considered high-risk, using a collaborative process to develop policies and procedures and to train personnel in their implementation.

Of particular concern is that the policies or procedures identify:

- How planning will occur
- The documentation required for the care team to work effectively
- Special consent considerations
- Monitoring requirements
- Special qualifications or skills of personnel involved in the care process

Clinical guidelines and pathways are frequently helpful and may be incorporated in the process. Monitoring provides the information needed to ensure that the policies and procedures are adequately implemented and followed for all relevant patients and services.

High risk patients and procedures include (where applicable):

- a) Care of emergency patients
- b) Management of patients in restraint (including the documentation required when a patient is restrained)
- c) Management of patients who may be a danger to themselves or others
- d) Management of the violent patient
- e) Management of the detoxification stage of treatment
- f) Care of frail, dependent and elderly patients
- g) The administration of electroconvulsive therapy
- h) The management of patients in seclusion (including the documentation required when a patient is treated in seclusion)
- i) Observation of patients treated for eating disorders

44.3.5 Criteria

44.3.5.1	Documented policies, clinical guidelines or standard operating procedures for identified high risk patients and procedures, which include items (a)-(i) in the standard intent above as a minimum, are available and readily accessible.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		

Guideline statement: The clinical and managerial leaders take responsibility for identifying the patients and services considered high risk, using a collaborative process to develop policies and procedures and training personnel in their implementation.
Implementation will be verified during the patient record audit.
The 40/80 rule for compliance will apply.

44.3.5.2	In-service training is provided to personnel to ensure they understand the intent and content of the policies and procedures.	Comment:
Severity: Serious		
Category: Patient Care		
NA NC PC C		



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Guideline statement: Where policies and procedures refer to patient care, compliance will be measured against the relevant areas in the patients' records during the record audit process. By implication, if the policies are implemented, personnel can be considered to have been trained and this criterion will be scored accordingly. This training could take the form of formal in-service training, introduction at orientation to policies, discussions at departmental meetings, case studies, etc.

44.3.5.3	Patients receive care consistent with the policies and procedures.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during patient record audit and patient interviews.		



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44.3.6 Standard

Adequate information is provided when obtaining informed consent from patients or their legal representatives.

Intent of 44.3.6

Patients and their families or decision-makers must receive adequate information to participate in care decisions by being informed as to what tests, procedures and treatments require consent and how they can give consent, for example verbally, by signing a consent form, or through some other mechanism. Patients and families understand who may, in addition to the patient, give consent.

Organisation policy must define the situations under which informed consent is required and specify the type of consent required, for example, written or verbal consent.

The leaders must agree and implement a standardised procedure for obtaining informed consent which meets all requirements of country-specific legislation and includes:

- a) Discussion of the patient's diagnosis and why the procedure is advised
- b) The identification and designation of the person who is authorised to perform the procedure
- c) A thorough explanation of the proposed procedure in a clear and understandable manner
- d) The expected benefits of the procedure
- e) The likelihood of success of the procedure
- f) The expected course of recovery following the procedure
- g) The potential risks and complications of the procedure
- h) A discussion of viable alternative options including risks and benefits
- i) The potential risks of refusing the proposed procedure
- j) Confirmation that the patient or their legal representative has understood the information provided
- k) An opportunity for the patient or their legal representative to ask questions
- l) Comprehensive documentation of the process followed to obtain informed consent.
- m) How to obtain consent if the patient is unable to give consent due to age, diminished mental capacity (for example, delirium, learning difficulties, etc.) or by virtue of their physical illness (for example, comatose)
- n) Documentation of the signature of the patient or their legal representative for written consent
- o) Documentation of verbal consent in the patient record

The procedure for obtaining informed consent should acknowledge and accommodate religious, cultural and social needs of patients and their families.

The leaders must agree and implement a process for the documentation of verbal consent. Where written consent is required, it can be useful to include the steps in the process in the consent form with space allocated for signature of the patient and the personnel member providing the information to confirm that the step has been completed satisfactorily.

Personnel responsible for obtaining informed consent must receive training on the agreed policies and procedures relating to informed consent to ensure that they are fully aware of organisation requirements.

Consent should only be obtained by suitably trained professional personnel who are familiar with the procedure and its risks, complications and alternatives. Ideally, the person who will perform the procedure should be the one to obtain consent for the procedure. Where this is not the case, the patient should be informed of which healthcare professional will be performing the procedure.

Where permissible according to country-specific legislation, organisation personnel who can consent on behalf of patients unable to give independent consent should be identified. This will require a detailed policy and procedure which should reflect the requirements set out in the relevant national legislation. A copy of this legislation should be available to be read in conjunction with the policy.



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44.3.6 Criteria

44.3.6.1	There is a documented process for the obtaining of informed consent.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: The documented process should outline all the steps in a) –o) in the standard intent above as a minimum. An assessment will be made of the provision of an organisational consent form and the completion of this form when indicated. This section will be scored NC if written consent is indicated but suitable forms are not used. When 1.3.6.2 is scored PC or NC because the available consent form has not been completed correctly, this criterion will be scored PC because the process has not been implemented.		

44.3.6.2 CRITICAL	Consent forms are completed comprehensively and available in the patient records.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: This section refers to all instances where patients give informed consent. If consent was not required for the patient records audited, this criterion becomes NA. If there is a correctly completed consent form the implication is that the patient has been provided with the information indicated in the relevant criterion above. Where the form is not completed in accordance with policy, this criterion will be scored PC or NC, depending on the average results of the record audit (40/80 rule will apply). The relevant criteria above will receive the same score. Compliance will be verified during the patient record audit.		

44.3.6.3 CRITICAL	Verbal consent is obtained and recorded according to organisational policy.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: This section refers to all instances where patients give verbal informed consent. If consent was not required for the patient records audited, this criterion becomes NA. If there is a correctly completed verbal consent form the implication is that the patient has been provided with the information indicated in the relevant criterion above. Where the form is not completed in accordance with policy, this criterion will be scored PC or NC, depending on the average results of the record audit (40/80 rule will apply). The relevant criteria above will receive the same score. Compliance will be verified during the patient record audit.		

44.3.6.4	Patients and their families or decision-makers receive adequate information in a clear and understandable way to enable them to participate in care decisions.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during patient record audit and patient interviews.		



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44.3.6.5	There is a written policy guiding the consent for HIV testing which is specific to patients in mental health care organisations.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: The presence of a mental health disorder does not automatically equal incapacity to consent to HIV testing. The policy must therefore address assessment for capacity to consent to HIV testing and the process to be followed if the patient is assessed as being incapable of giving informed consent.		

44.4 Medication Management

44.4.1 Standard

Medication is ordered according to organisational policy and stored in a secure and clean environment.

Intent of 44.4.1

Organisational policy on the ordering and storage of medication must be followed in each patient care area which stores medication for administration to patients under their care. Each organisation must identify those individuals permitted to order medication. These individuals must have the knowledge and experience required by law, registration or regulations to be permitted to order medication. The organisation must identify any additional individuals permitted to order medication in emergency situations.

Medication must be stored in a clean and secure environment that complies with legislation, regulation and professional practice standards.

Medication must be clearly labelled, correctly stored and protected from heat, light and moisture where necessary. Scheduled (controlled) drugs/narcotics/barbiturates and other dangerous drugs must be stored according to country-specific requirements. As a minimum, these drugs must be stored in a locked cupboard or container of substantial construction. "Substantial" construction is interpreted to be a cupboard/container that is mounted to the wall or fixed to the floor. Most often, these are steel cabinets, but solid wooden cupboards are also acceptable. There must be a registry, log or other system to monitor and account for controlled substances, which must be completed at the same time that medication is received, administered or returned.

Refrigeration facilities must be provided for safe storage of certain medication. There must be a process to ensure that thermolabile medication has been stored and transported at the correct temperature throughout the life of the medication. Refrigerators must be cleaned regularly and defrosted when necessary. Doors, hinges and seals must all be functional.

44.4.1 Criteria

44.4.1.1	Medication is ordered according to organisational policy.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: The organisational policy for ordering medication should be readily available to unit personnel. Documentation relating to the ordering of medication will be assessed for compliance to the organisational policy.		



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44.4.1.2	All storage areas for medication and pharmaceutical supplies comply with current pharmaceutical acts and regulations and manufacturer guidelines (for example, security, temperature, light and humidity control).	Comment:
Severity: Very Serious		
Category: Legality		
NA NC PC C		
Guideline statement: Note must be made of provision for medication that should not be exposed to direct sunlight. Where the ambient temperature in the cupboard/room cannot be maintained at 20-25 degrees Celsius there should be evidence that the environmental temperature is monitored and complies with the manufacturer's requirements for medication storage.		

44.4.1.3	Medication is stored in a locked storage device or cabinet that is accessible only to authorised personnel.	Comment:
Severity: Serious		
Category: Efficiency		
NA NC PC C		
Guideline statement: All medication storage areas (trolleys, cupboards, rooms, refrigerators etc.) will be examined for compliance. An unlocked refrigerator will only be accepted if it is in a locked room with access limited to relevant personnel only. An unlocked refrigerator in an unlocked room will be scored as NC.		

44.4.1.4	Medication identified for special control by legislation or organisational policy is stored in a cabinet of substantial construction, for which only authorised personnel have the keys.	Comment:
Severity: Very Serious		
Category: Legality		
NA NC PC C		
Guideline statement: Country-specific laws and regulations or organisational policy will determine the nature of such medication.		

44.4.1.5 CRITICAL	Medication identified for special control by legislation or organisation policy is accurately accounted for.	Comment:
Severity: Very Serious		
Category: Legality		
NA NC PC C		
Guideline statement: Compliance will be measured against the local medication control regulations. Control measures generally include the keeping of registers for these items.		

44.4.1.6 CRITICAL	Medication is securely and legibly labelled with relevant information as required by legislation and organisational policy.	Comment:
Severity: Very Serious		
Category: Legality		
NA NC PC C		
Guideline statement: Country-specific regulations apply, but must include at least the following: • Name of ward and/or name of patient and patient record number (if applicable) • Proprietary name/approved name or name of each active ingredient • Direction with regard to the manner in which medication must be used • Number of dose units in the container • Date of dispensing • Expiry date and batch number • Additional labels with warnings and storage instructions in accordance with organisation policy		



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44.4.1.7	Medication is stored in a clean environment.	Comment:
Severity: Serious		
Category: Pat & Staff Safety		
NA NC PC C		
Guideline statement: This applies to all medication storage areas in the organisation.		

44.4.1.8	A dedicated refrigerator is available for medication requiring storage at low temperatures.	Comment:
Severity: Very Serious		
Category: Efficiency		
NA NC PC C		
Guideline statement: There should be a refrigerator designated for the storage of medication only.		

44.4.1.9	The temperature of the refrigerator is monitored and recorded according to organisational policy.	Comment:
CRITICAL		
Severity: Very Serious		
Category: Efficiency		
NA NC PC C		
Guideline statement: Documented evidence in the form of daily temperature recording and monitoring of the refrigerator/s must be available. The temperature must be maintained within a safe range according to manufacturer's specifications. This is usually 2-8 degrees Celsius.		

44.4.1.10	Appropriate action is taken and recorded when the temperature of the refrigerator is outside the recommended range.	Comment:
Severity: Very Serious		
Category: Efficiency		
NA NC PC C		
Guideline statement: The actions taken must be documented and kept on file for reference purposes. Monitoring logs should demonstrate more frequent temperature checks until it is confirmed that the action taken has been successful in restoring maintenance of the required refrigerator temperature range.		

44.4.1.11	Expiry dates (including those of emergency medication) are checked regularly at defined intervals according to organisational policy and medication is replaced before the expiry date.	Comment:
Severity: Very Serious		
Category: Efficiency		
NA NC PC C		
Guideline statement: Compliance will be measured against relevant laws and organisation policies.		



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44.4.2 Standard

Medication use throughout the organisation complies with applicable laws and regulations.

Intent of 44.4.2

Medication management is not only the responsibility of the pharmaceutical service but also of managers and clinical care providers. Medical, nursing, pharmacy and administrative personnel should participate in a collaborative process to develop and monitor policies and procedures. Each organisation must identify those individuals permitted to prescribe medication. These individuals must have the requisite knowledge and experience, and be permitted by law, registration or regulations to prescribe medication. The organisation must identify any additional individuals permitted to prescribe medication in emergency situations. Prescription of medication and verbal medication orders must be issued and documented according to organisational policy.

The safe administration of medication requires a strict and comprehensive policies. The policy relating to the administration of medication into catheters and tubing must include steps to ensure that the tubing is checked prior to administration to ensure that medication is not administered via the wrong route. Only personnel who are suitably trained and experienced must be permitted to administer medication to patients and this responsibility must be documented.

The patient, medical practitioner, nurse and other care providers must work together to monitor patients on medication. The purpose of monitoring is to evaluate the patient's response to medication, adjust the dosage or type of medication when needed and identify, record and report any adverse effects the patient may suffer as a result of the medication.

Doctors, nurses and pharmacists are expected to report adverse reactions that are suspected to be related to medication, irrespective of whether the reaction is well recognised, potentially serious or clinically insignificant. The organisation must ensure that country-specific requirements for the reporting of adverse drug reactions are followed.

In addition to the reporting of adverse drug reactions, there must be a system for the reporting and investigation of medication errors. The system must focus on the prevention of medication errors through understanding the types of errors that occur. Each error should be investigated to understand why it happened. Where necessary, processes should be adapted to incorporate measures which will reduce the likelihood of the error happening again. Personnel training, either in existing or adapted medication administration processes, can be beneficial in preventing future errors. Pharmacy personnel should participate in such training interventions.

Only personnel who are suitably trained and experienced may administer medication to patients. The responsibility of these persons for medication administration is documented.

44.4.2 Criteria

44.4.2.1 Severity: Very Serious Category: Patient Care NA NC PC C	Policies and procedures that guide the safe prescribing and administration of medication are implemented.	Comment:
Guideline statement: These policies should be developed by all role-players i.e. medical practitioners, nursing personnel and pharmacists. Country-specific regulations will apply. Compliance will be verified during the patient record audit.		



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44.4.2.2	Only those permitted by the organisation and by relevant laws and regulations prescribe medication.	Comment:
Severity: Very Serious		
Category: Legality		
NA NC PC C		
Guideline statement: Compliance will be verified during the patient record audit.		

44.4.2.3	Only those permitted by the organisation and by relevant laws and regulations administer medication.	Comment:
Severity: Very Serious		
Category: Legality		
NA NC PC C		
Guideline statement: Compliance will be verified during the patient record audit.		

44.4.2.4	There is a process to place limits, when appropriate, on the prescribing or ordering practices of individuals.	Comment:
Severity: Very Serious		
Category: Efficiency		
NA NC PC C		
Guideline statement: This is particularly relevant where certain medication may only be prescribed by designated specialists. Compliance will be verified with reference to organisational policy and country-specific legislation.		

44.4.2.5	Relevant personnel are trained in correct prescribing, ordering and administration practices.	Comment:
Severity: Serious		
Category: Efficiency		
NA NC PC C		
Guideline statement: Compliance will be verified during personnel interviews and training records in personnel files.		

44.4.2.6	On admission, all current medication taken by the patient is documented in the patient record, including herbal and over-the-counter medication.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Implementation of the organisational policy is required. This includes traditional and homeopathic medication. These should be re-prescribed on the prescription sheet if indicated. Compliance will be verified during the patient record audit.		

44.4.2.7	Verbal and telephonic medication prescriptions are documented according to organisational policy.	Comment:
Severity: Very Serious		
Category: Legality		
NA NC PC C		



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Guideline statement: The procedure to be followed must be clearly documented in line with current legislation and implemented.
Compliance will be verified during the patient record audit.

44.4.2.8	There is evidence that patients are identified before medication is administered.	Comment:
Severity: Very Serious		
Category: Pat & Staff Safety		
NA NC PC C		

Guideline statement: This will be assessed in relation to the existing policy framework, including the use of patient identification bands when required. Compliance will also be confirmed while observing medication administration practices and during personnel interviews.

44.4.2.9 CRITICAL	Medication is checked against the original prescriptions and administered as prescribed.	Comment:
Severity: Very Serious		
Category: Pat & Staff Safety		
NA NC PC C		

Guideline statement: Compliance could be assessed by observing the administration of medication while visiting the clinical departments.
During the patient record audit process, check for evidence of transcribing by nurses from the original prescription onto the medication administration sheet.

44.4.2.10	The medication prescribed for and administered to each patient is recorded.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		

Guideline statement: Compliance will be verified during the patient record audit.

44.4.2.11	Healthcare professionals monitor medication effects on patients collaboratively.	Comment:
Severity: Serious		
Category: Patient Care		
NA NC PC C		

Guideline statement: Patient record audits should reveal whether the effect of medication was monitored, recorded and brought to the attention of the medical practitioner. The incidence and intensity of therapeutic and undesirable effects should be monitored. While clinical signs and symptoms are useful measures of drug therapy efficacy, additional information may be required, for example, plasma drug concentration may be used in the initiation and maintenance of certain medication.

44.4.2.12	Therapeutic plasma medication concentrations are checked in accordance with manufacturers' guidelines and organisational policy.	Comment:
Severity: Serious		
Category: Patient Care		
NA NC PC C		

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Guideline statement: While clinical signs and symptoms are useful measures of medication therapy efficacy, additional information may be required by utilising plasma medication concentration levels during the initiation and maintenance of certain medication.
The organisational policy, manufacturer's guidelines and medical practitioner's orders identify those instances in which therapeutic medication levels must be obtained.
Compliance is verified during the patient record audit.

44.4.2.13	Adverse drug reactions (ADR) are observed, recorded and reported through a process and within a time frame defined by the organisation.	Comment:
Severity: Very Serious		
Category: Legality		
NA NC PC C		

Guideline statement: An adverse drug reaction (ADR) is an expression that describes harm associated with the use of given medication at a normal dose. The meaning of this expression differs from the meaning of 'side effect', as this last expression might also imply that the effects can be beneficial. The study of ADRs is the concern of the field known as pharmacovigilance.
Compliance is verified during the patient record audit.
Evidence will be sought of documentation reporting the reaction to the physician and the pharmacist.

44.4.2.14	Medication errors are reported through a process and within a time frame defined by the organisation.	Comment:
Severity: Very Serious		
Category: Pat & Staff Safety		
NA NC PC C		

Guideline statement: A process must be in place for the recording of medication errors. These include errors in relation to prescribing, dispensing and administration of medication.



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44.5 Food and Nutrition Therapy

44.5.1 Standard

Food and nutrition therapy appropriate for the patient and consistent with his or her clinical care is regularly available.

Intent of 44.5.1

A qualified caregiver orders appropriate food or other nutrients. The patient participates in planning and selecting foods, and the patient's family may, when appropriate, participate in providing food. They are educated as to which foods are contraindicated, including information about any medication associated with food interactions. When possible, patients are offered a variety of food choices consistent with their nutritional status. The nutritional status of the patients is monitored.

44.5.1 Criteria

44.5.1.1	Food appropriate to the patient is regularly available.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified from interviews with personnel and patients, especially regarding times of meals and the availability of special diets. Menus and meal times will be checked.		

44.5.1.2	Food orders appropriate for the patient's nutritional status and needs are documented in the patient's record.	Comment:
Severity: Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during the patient record audit, especially as it relates to the patient's assessment, the care plan and the identification of special diet requirements. Prescribed diets must be ordered and delivered to the patient.		

44.5.1.3	Wherever possible, patient food preferences are respected and substitutions made available.	Comment:
Severity: Moderate		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during patient interviews.		

44.5.1.4	Patient food preferences consider cultural and religious requirements.	Comment:
Severity: Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during patient interviews and with reference to cultural and religious requirements identified during the initial assessment of the patient.		



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44.5.1.5	When families provide food, they are educated about the patient's dietary limitations.	Comment:
Severity: Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during the patient record audit.		

44.5.1.6	Patients assessed as being at nutritional risk receive nutrition therapy.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Such patients could include those unable to take normal meals for example, unable to swallow, specific disease processes, etc. This could include the provision of nutritional supplements or specialised diets. This criterion and those that follow apply only to nutrition therapy. Compliance of all these criteria will be verified during the patient record audit.		

44.5.1.7	A collaborative process is used to plan, deliver and monitor nutrition therapy.	Comment:
Severity: Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: This should include the involvement of a dietician if available. Compliance will be verified during the patient record audit.		

44.5.1.8	Nutrition therapy provided, whether oral, enteral or parenteral, is documented in the patient's record.	Comment:
Severity: Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during the patient record audit.		

44.5.1.9	Response to nutrition therapy is monitored and recorded.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: This may take the form of weight/mass checks, laboratory tests for protein and/or lipid levels etc.		



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44.6 Patient and Family Education

44.6.1 Standard

Each patient's educational needs are assessed and written in their record.

Intent of 44.6.1

Education is focused on the specific knowledge and skills the patient and his or her family will need to make care decisions, participate in care, and continue care at home. Variables like educational literacy, beliefs and limitations are taken into account.

The organisation decides the placement and format for educational assessment, planning and delivery of information in the patient's record.

Education is provided to support care decisions of patients and families. In addition, when a patient or family directly participate in providing care, for example changing dressings, feeding and administration of medication, they need to be educated.

On occasion, it is important that they be aware of any financial implications associated with care choices, such as choosing to remain an inpatient rather than an outpatient.

Education in areas that carry high risk to patients is routinely provided by the organisation, for instance safe and effective use of medications and medical equipment.

Community organisations that support health promotion and disease prevention education are identified, and, when possible, ongoing relationships are established.

44.6.1 Criteria

44.6.1.1	Patients and families' educational needs are assessed and recorded.	Comment:
Severity: Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during the patient record audit.		

44.6.1.2	There is a uniform process for the recording of patient education information.	Comment:
Severity: Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during the patient record audit.		

44.6.1.3	Patients and families are educated about participation in the care process.	Comment:
Severity: Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during patient interviews and patient record audits.		



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44.6.1.4	Patients and families are informed about the financial implications of care decisions.	Comment:
Severity: Moderate		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during patient interviews.		

44.6.1.5	Patients and their families (when required) are educated about their diagnosis and relevant health risks.	Comment:
Severity: Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: For example, safe use of medication and medical equipment, medication and food interaction, therapeutic diet and food interactions, defaulting on medication use, etc. Compliance will be verified during patient interviews and patient record audits.		

44.6.1.6	The organisation identifies and establishes relationships with community resources that support continuing mental and physical health promotion and disease prevention education.	Comment:
Severity: Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Evidence of meetings will be required.		

44.6.1.7	Patients and families are referred to these organisations as appropriate.	Comment:
Severity: Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during patient interviews and patient record audits		



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44.6.2 Standard

Education methods consider the patient's and family's values and preferences.

Intent of 44.6.2

Learning occurs when attention is paid to the methods used to educate patients and families. The organisation selects appropriate educational methods and people to provide the education.

Personnel collaboration helps to ensure that the information patients and families receive is comprehensive, consistent, and as effective as possible.

44.6.2 Criteria

44.6.2.1	The patient and family are taught in a language and format that they can understand.	Comment:
Severity: Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during patient interviews and patient record audits.		

44.6.2.2	Those who provide education have the knowledge and communication skills for effective education.	Comment:
Severity: Serious		
Category: Efficiency		
NA NC PC C		
Guideline statement: Compliance will be verified during patient interviews and personnel training records.		

44.6.2.3	Health professionals caring for the patient work collaboratively when appropriate.	Comment:
Severity: Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during the patient record audit.		

44.6.2.4	Interaction between personnel, the patient and family is noted in the patient's record.	Comment:
Severity: Moderate		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during the patient record audit.		



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44.7 Continuity of Care

44.7.1 Standard

The organisation designs and implements processes to provide continuity of patient care services within the organisation and coordination among health professionals.

Intent of 44.7.1

As patients move through a healthcare organisation from admission to discharge or transfer, several departments, services and healthcare providers may be involved in providing care. Without coordination and effective transfer of information and responsibilities, errors of omission and commission are more likely to occur, exposing the patient to avoidable risks. The organisation should therefore document and implement procedures to minimise the likelihood of these errors occurring.

44.7.1 Criteria

44.7.1.1 Severity: Serious Category: Patient Care NA NC PC C	Policies and procedures that guide the movement of patients within the organisation are implemented.	Comment:
Guideline statement: This refers to transfers between departments within the organisation as well as admissions from the out-patient department. Please take note that a transfer to theatre/procedure room is also regarded as an internal transfer. Compliance will be verified during the patient record audit.		

44.7.1.2 Severity: Serious Category: Patient Care NA NC PC C	Individuals responsible for the patient's care and its coordination are identified for all phases of patient care.	Comment:
Guideline statement: Compliance will be verified during the patient record audit.		

44.7.1.3 Severity: Serious Category: Patient Care NA NC PC C	Continuity and coordination are evident throughout all phases of patient care.	Comment:
Guideline statement: Compliance will be verified during the patient record audit.		

44.7.1.4 Severity: Serious Category: Patient Care NA NC PC C	When a patient is transferred within the organisation, they are accompanied by their patient record.	Comment:
Guideline statement: Compliance will be verified during the patient record audit.		



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44.7.1.5	Patient handover between healthcare professionals is standardised according to organisational policy.	Comment:
Severity: Very Serious		
Category: Pat & Staff Safety		
NA NC PC C		

Guideline statement: The complexity and comprehensiveness of the content of handover information required to achieve continuity of care will differ for different types of handover situations. The details to be included in each situation should be agreed upon by all stakeholders and formalised in the organisational policy framework to ensure a standardised approach at all times. The transfer of information can be verbal for low risk situations, but should be documented, preferably by the use of forms, for high risk situations. Compliance will be verified during patient record audit.

44.7.2 Standard

There is a process known to personnel to refer patients for specialised consultation, investigations and/or treatment at other healthcare organisations.

Intent of 44.7.2

Medical practitioners may need to refer patients to other facilities for a secondary consultation to confirm an opinion, access more extensive diagnostic evaluations than may be available locally, or access specialised treatment that the referring organisation may be unable to provide.

This standard refers to the process by which a patient is referred to another healthcare organisation only to access services not offered by the referring organisation and then returns to the referring organisation to receive ongoing care, either as part of their care as an inpatient (in which case they will return to the organisation following their review at the referral organisation) or while under the care of the outpatient department. Organisational policy must clearly describe this referral process.

44.7.2 Criteria

44.7.2.1	Policies and procedures that guide the movement of patients for referral to another organisation are implemented.	Comment:
Severity: Serious		
Category: Patient Care		
NA NC PC C		

Guideline statement: The organisation must clearly describe the referral process, especially where patients are sent to another healthcare organisation for specialist consultation or special investigations and then return to the referring organisation. Note that this criterion does not refer to the transfer of a patient to another healthcare organisation to take over the care of the patient. Compliance will be verified during the patient record audit.

44.7.2.2	Patients are accompanied and monitored by an appropriately qualified person during transfer.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		

Guideline statement: This should include the cadre of personnel suitable for the level of care required by the patient to be transferred.



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44.7.2.3	A copy of the referral note is available in the patient record.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during patient record audit.		

44.7.2.4	Follow-up care based on the findings of investigations and/or consultations performed outside the referring organisation is documented in the patient record.	Comment:
Severity: Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during patient record audit.		

44.7.3 Standard

There is a process to transfer patients to another organisation to meet their continuing needs.

Intent of 44.7.3

This standard refers to the process by which a patient is discharged from the transferring organisation and handed over to receive ongoing care at another healthcare organisation.

Transfer may be to a higher level of care for specialised consultation and/or treatment, or for less intensive services such as sub-acute care or long-term rehabilitation. To ensure continuity of care, adequate information must accompany the patient.

Transfer may be an uncomplicated process with the patient alert and talking, or may involve continuous nursing or medical supervision. The qualifications of the individual accompanying the patient must be appropriate to the level of care required during transfer. The process for transferring the patient must consider transportation needs as well as clinical needs.

44.7.3 Criteria

44.7.3.1	There is a documented process for transferring patients to other organisations.	Comment:
Severity: Very Serious		
Category: Pat & Staff Safety		
NA NC PC C		
Guideline statement: The documented process should be available in the unit and readily accessible to unit personnel. The process must include: • Guidance on how to make the necessary transport arrangements • The qualifications required in the clinical personnel accompanying the patient for various levels of clinical care		



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44.7.3.2	When a patient is transferred to another organisation, the receiving organisation is given a written summary of the patient's clinical condition and the interventions performed by the referring organisation.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during patient record audit.		

44.7.3.3 CRITICAL	A copy of the transfer summary is available in the patient record.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during patient record audit.		

44.7.3.4 CRITICAL	The transferring organisation documents the transfer in the organisation's patient record.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: • Documentation must include as a minimum: • The reason(s) for the transfer • Any special conditions related to transfer • The condition of the patient before transfer • The name of the healthcare organisation receiving the patient Compliance will be verified during patient record audit.		

44.7.3.5	The referring organisation determines that the receiving organisation can meet the patient's continuing care needs, and establishes arrangements to ensure continuity.	Comment:
Severity: Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: This should be done by the departmental management team when referral pathways for the unit are established. These pathways should be documented along with all relevant details, for example, transport arrangements and contact details. The documented pathway should be readily available to personnel who will be responsible for making the decision to transfer patients to another organisation.		

44.7.3.6	Patients are accompanied and monitored by an appropriately qualified person during transfer.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: This should include the cadre of personnel suitable for the level of care required by the patient to be transferred.		



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44.7.4 Standard

There is an organised process to discharge patients.

Intent of 44.7.4

Care planning should include arrangements to meet the patients' continuing needs after discharge as early in the care process as possible. Instructions for discharge and follow-up visits must be clear and provided in writing.

The discharge summary is one of the most important documents for ensuring continuity of care and facilitating correct management at subsequent visits. Information provided by the organisation may include when to resume daily activities, preventive practices relevant to the patient's condition and, when appropriate, information on coping with disease or disability. At discharge, patients should be alerted to symptoms and signs related to their diagnosis which require urgent medical attention and how to access healthcare services should they develop such symptoms or signs. This process is referred to as "safety-netting".

The discharge summary must contain the following content as a minimum:

- The diagnosis of main and significant illnesses and reason for admission
- The results of investigations that will influence further management
- All procedures performed
- All significant findings
- The patient's condition at discharge
- Discharge medication
- Follow-up arrangements where appropriate, including emergency review

44.7.4 Criteria

44.7.4.1	There is a documented process to discharge patients.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: This process must include any criteria or conditions that the patient and his/her family need to meet prior to discharge. The documented process must be available in the unit and readily accessible to, and known by organisational personnel.		

44.7.4.2	Where the initial assessment indicates that discharge planning will be required, planning requirements are included in the patient's care plan and completed as scheduled.	Comment:
Severity: Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during patient record audits.		

44.7.4.3	The organisation works with the family, caregivers, healthcare practitioners and agencies outside the organisation to ensure timely and appropriate discharge and continuity of care.	Comment:
Severity: Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during patient record audits.		



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44.7.4.4	Patients and where appropriate their families or caregivers are given understandable follow-up instructions which are documented in the patient's record.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		

Guideline statement: The follow-up instructions may be provided to the patient by the medical practitioner, or by the nurse following instructions from the doctor.
The instructions must include as a minimum:

- Medication information
- Possible complications
- Return for follow-up care
- When and where to obtain urgent care

Compliance will be verified through patient record audits and patient interviews.

44.7.4.5	A discharge summary, which includes (a)-(g) in the standard intent above as a minimum, is written by the medical practitioner when each patient is discharged.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		

Guideline statement: This criterion and the next apply to the discharge summary completed by the medical practitioner and not to the nurses' discharge note. Where a discharge summary has only been partially completed i.e. does not include all items (a)-(f) it will be scored PC. A nurse's discharge note is not acceptable and is scored NC.
The 40/80 rule for compliance will apply.
Compliance will be verified during patient record audit.

44.7.4.6	Each record contains a copy of the discharge summary.	Comment:
Severity: Serious		
Category: Patient Care		
NA NC PC C		

Guideline statement: This will ensure that the discharge summary is available for follow-up visits.
Compliance will be verified during patient record audit.

44.7.4.7	When appropriate the patient is given a copy of the discharge summary.	Comment:
Severity: Moderate		
Category: Patient Care		
NA NC PC C		

Guideline statement: This could be appropriate if the patient's care will be continued outside of the country.



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44.8 Quality Improvement

44.8.1 Standard

A formalised proactive quality improvement approach is maintained in the rehabilitation service.

Intent of 44.8.1

The senior management team is responsible for ensuring that standards are set throughout the organisation. Within each department or service, unit managers should ensure that standards are set for the particular unit. Departmental or service managers should use available data and information to identify priority areas for quality monitoring and improvement. This should be done in collaboration with the organisation's central quality management structures to ensure coordinated quality improvement activities throughout the organisation.

Quality monitoring is typically applied to high risk, high volume or high cost activities, or areas of concern identified by personnel, patients or visitors. Some examples of activities that may benefit from quality monitoring include:

- Patient assessment
- Procedures carried out
- The use of medication
- Medication errors
- Patient and family expectations and satisfaction

The following will be evaluated:

- The manner in which problems were identified and prioritised in this service for which quality improvement activities were initiated
- The processes put in place to resolve the problems
- The identification of indicators to measure improvement
- The tool(s) used to evaluate these indicators
- The monitoring of these indicators and corrective steps taken when goals were not achieved
- Graphed and/or tabled results, as appropriate

A once-off project such as acquiring a specific item of equipment will be scored NC. Quality improvement processes not related to the clinical quality of patient care but to the environment within which care is provided, for example, monitoring the checking of the emergency trolley over time, will be scored PC.

44.8.1 Criteria

44.8.1.1 Severity: Very Serious Category: Efficiency NA NC PC C	There is a written quality improvement programme for the rehabilitation service that is developed and agreed upon by the personnel of the service.	Comment:
Guideline statement: Root Criterion: Guideline Statement: The minimum requirement for compliance will be the availability of evidence of: • At least one clinical audit • Participation in documentation (patient record) audits • Participation in the monitoring of near misses, adverse events and sentinel events		



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44.8.1.2	Indicators of performance are identified to evaluate the quality of treatment and patient care.	Comment:
Severity: Very Serious		
Category: Efficiency		
NA NC PC C		

Guideline statement: As the emphasis is on the quality of patient care, this cannot be compliant if there is no evidence of clinical audits. These could take the form of evaluation of the implementation of clinical guidelines, case discussions, reports on negative incidents, etc. An indicator, in this context, is a measure used to determine improvements in clinical care over time. The information gained must be used to identify and implement improvement processes. Statistical reports without documented evidence of continuous quality improvement will be scored PC.

44.8.1.3	Processes are selected in order of priority for evaluation and improvement in the quality of treatment and care.	Comment:
Severity: Serious		
Category: Efficiency		
NA NC PC C		

Guideline statement: Quality monitoring is typically applied to high risk, high volume or high cost activities, or areas of concern identified by personnel, patients or visitors.

44.8.1.4	The quality improvement cycle includes the monitoring and evaluation of the standards set, and remedial action implemented.	Comment:
Severity: Very Serious		
Category: Efficiency		
NA NC PC C		

Guideline statement: The quality improvement cycle does not stop with the implementation of a quality improvement plan. Ongoing monitoring of the process or procedure under consideration is required, with evaluation of the monitoring data collected to ensure that the plan has achieved its intended aim. If not, the quality improvement cycle should be repeated, incorporating the learning from previous attempts, until the desired aim is achieved.

44.8.1.5	A documentation audit system is in place.	Comment:
Severity: Very Serious		
Category: Efficiency		
NA NC PC C		

Guideline statement: Documented evidence of such audits must be provided at departmental level. Where the documentation (patient record) audit is an organisation-wide multidisciplinary process, which includes all clinical departments, this criterion will be scored according to the findings of the involvement of each department in the process. Evaluation of results and remedial action taken must be documented as well as maintenance of achieved improvements over time.

44.8.1.6	An incident managing system for monitoring near misses/adverse events/sentinel events is implemented, which includes the documentation of responses to recorded incidents and interventions to prevent recurrence of the incident or minimise harm in the event of a recurrence.	Comment:
Severity: Very Serious		
Category: Efficiency		
NA NC PC C		



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Guideline statement: Participation at department level in the organisation's overall system of the monitoring of near misses/adverse events/sentinel events will be evaluated. Analysed data, with responses/remedial action related to departmental incidents are required.

44.9 Patient Rights

44.9.1 Standard

The organisation is responsible for providing processes that support patient and family rights during care.

Intent of 44.9.1

This refers to the implementation of organisational policy on patient and family rights. Compliance will be verified during observation of patient care processes, patient record audits and patient interviews.

44.9.1 Criteria

44.9.1.1	There are processes that support patient and family rights during care.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: The organisation's policies will be evaluated against country-specific legislation, where applicable. The implementation thereof will be evaluated during the auditing of patient records, patient interviews and observation during a walk-about in the department. This applies to all the relevant criteria below.		

44.9.1.2	There are processes to ensure that care is respectful of the patient's personal values and beliefs.	Comment:
Severity: Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Implementation will be evaluated during the auditing of patient records, patient interviews and observation during a walk-about in the department.		

44.9.1.3	Measures are taken to protect the patient's privacy, person and possessions.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Confidentiality of patient information and patient privacy must be protected at all times. Standard operating procedures specific to the conditions and processes in the unit must be in place, known to relevant personnel and implemented. Likewise, security measures specific to the unit must be in place to safeguard the patients' person and possessions.		



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44.9.1.4	Personnel respect the rights of patients and families to treatment and to refuse treatment.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: Compliance will be verified during patient record audit and patient interviews.		

44.9.1.5	Patients are informed of their right not to donate human tissue or to participate in research.	Comment:
Severity: Very Serious		
Category: Patient Care		
NA NC PC C		
Guideline statement: It is very important to ensure that this internationally accepted right is implemented in the mental health patient population.		

44.10 Prevention and Control of Infection

44.10.1 Standard

There is a comprehensive infection prevention and control programme encompassing patient care and employee health.

Intent of 44.10.1

This standard measures the implementation of the organisational policy for infection prevention and control at unit level as well as the identification of unit-specific infection control risks. Personnel must be aware of the policy and there must be evidence that it is implemented throughout the organisation.

44.10.1 Criteria

44.10.1.1	There is a programme, which is implemented, to reduce the risks of healthcare associated infections in patients and healthcare workers.	Comment:
Severity: Very Serious		
Category: Pat & Staff Safety		
NA NC PC C		
Guideline statement: • The programme must include as a minimum: • Prevention of the spread of respiratory tract infections • Prevention of the spread of urinary catheter related infections • Prevention of the spread of infection through surgical wounds • Prevention of the spread of infection through intravascular/invasive devices • Safe injection practices, including single-use injection devices • Care of the infected patient • The wearing of personal protective equipment • The disinfection of equipment • Management of infected/soiled linen • Management of hazardous waste		



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44.10.1.2	The department participates in the overall programme for quality management and improvement of infection control.	Comment:
Severity: Very Serious		
Category: Pat & Staff Safety		
NA NC PC C		
Guideline statement: Compliance will be measured by evidence of departmental representation at organisational infection control meetings and activities and by the departmental contribution of data to the organisational infection control data collection systems.		

44.10.1.3	The department identifies the procedures and processes associated with the risk of infection, and implements strategies to reduce risk.	Comment:
Severity: Very Serious		
Category: Pat & Staff Safety		
NA NC PC C		
Guideline statement: The department participates in, and has documented evidence of, the identification of risks in their department. The strategies to reduce risk should be defined in policies and procedures and include the risks listed in this standard as a minimum. Such documentation must form part of the organisation-wide infection control processes.		

44.10.1.4	Personnel are trained in correct hand washing procedures.	Comment:
Severity: Very Serious		
Category: Pat & Staff Safety		
NA NC PC C		
Guideline statement: Documented evidence of personnel training will be required, together with evidence of monitoring of implementation, for example, hand washing audits performed regularly according to organisational policy.		

44.10.1.5	Organisational policy on handling, storage and disposal of healthcare waste is implemented.	Comment:
Severity: Very Serious		
Category: Pat & Staff Safety		
NA NC PC C		
Guideline statement: The implementation of the organisation's master policy will be evaluated in the department. The processes must include handling at source, collection, storage and disposal of healthcare waste.		

44.10.1.6	Individuals who collect specimens are trained in the proper collection and handling of microbiological specimens.	Comment:
Severity: Very Serious		
Category: Pat & Staff Safety		
NA NC PC C		
Guideline statement: Documented evidence of personnel training will be required.		



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44.10.1.7	The department provides education on infection control practices to personnel, doctors, patients and, as appropriate, family and other caregivers.	Comment:
Severity: Very Serious		
Category: Pat & Staff Safety		
NA NC PC C		
Guideline statement: Documented evidence of personnel training will be required. Information pamphlets, posters, individualised education etc. will provide evidence of patient, family and other caregiver education		

44.11 Occupational Health and Safety and Risk Management

44.11.1 Standard

The organisation directs and controls risk management programmes, which include health and safety programmes that comply with legislation.

Intent of 44.11.1

This standard measures the implementation of the organisational policy for occupational health and safety and risk management at unit level as well as the identification of unit-specific risks. Personnel must be aware of the policy and there must be evidence that it is implemented throughout the organisation.

44.11.1 Criteria

44.11.1.1	The department conducts ongoing monitoring of risks through documented assessments as part of organisation's risk management processes.	Comment:
Severity: Very Serious		
Category: Pat & Staff Safety		
NA NC PC C		
Guideline statement: The department participates in and has documented evidence of the identification of risks in their department. Such documentation must form part of the organisation-wide risk management processes. All relevant aspects and services of the department, for example, patient, personnel and visitor related risks, financial and legal risks, physical facilities, security and environmental risks, etc. should be included.		

44.11.1.2	The Health and Safety representative for the department supervises implementation of the organisation's health and safety programme.	Comment:
Severity: Serious		
Category: Pat & Staff Safety		
NA NC PC C		
Guideline statement: Documented evidence is required.		

44.11.1.3	Fire safety measures are implemented.	Comment:
Severity: Very Serious		
Category: Pat & Staff Safety		
NA NC PC C		



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Guideline statement: The department's participation in the organisation's overall fire safety processes will be evaluated.

44.11.1.4	Relevant personnel are trained in the procedures relating to the reporting and investigation of near misses/adverse events/sentinel events.	Comment:
Severity: Serious		
Category: Efficiency		
NA NC PC C		

Guideline statement: Documented evidence of training required.

44.11.1.5	Security measures are implemented to ensure the safety of patients, personnel and visitors.	Comment:
Severity: Very Serious		
Category: Pat & Staff Safety		
NA NC PC C		

Guideline statement: The department's participation in the organisation's overall security plan for personnel and visitors will be evaluated.

44.11.1.6	Where vulnerable patients are cared for, special safety and security measures are implemented in accordance with organisational policy.	Comment:
Severity: Very Serious		
Category: Pat & Staff Safety		
NA NC PC C		

Guideline statement: Some method of access control into the unit will be required for compliance with this criterion. Alarm systems can include intruder detection systems, security guards and panic alarms. Security guards must be monitored to ensure that they are providing an effective access control system. Alarm systems may be replaced by CCTV.

44.11.1.7	All personnel are trained regarding their role in providing a safe and secure patient care facility.	Comment:
Severity: Very Serious		
Category: Pat & Staff Safety		
NA NC PC C		

Guideline statement: This is of particular relevance to the mental health care setting where all personnel must be alert to the safety and security of vulnerable patients. Documented evidence of training required.